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Influenza: Protecting Individuals and Communities through Prevention, Early Recognition and Surveillance - II

This is the second article of two in a series on "Influenza: Protecting Individuals and Communities through Prevention, Early Recognition and Surveillance"

Recognizing influenza symptoms

Influenza commonly presents with sudden onset of fever, cough, sore throat, runny or blocked nose, headache, muscle aches and marked tiredness. Symptoms usually develop one to four days after infection. Although most people recover without specific treatment, the cough and fatigue may persist beyond the initial illness [1]. Influenza cannot always be distinguished from other respiratory infections on symptoms alone. Individuals should avoid self-diagnosis and seek medical advice when symptoms are severe, persistent or worsening, particularly if they belong to a high-risk group.

Who is at increased risk?

Anyone can develop influenza, but certain groups are more likely to experience complications. These include children younger than five years, older adults, pregnant women, people with chronic conditions such as asthma, heart disease and diabetes, and individuals with weakened immune systems. Healthcare workers may also have increased exposure to infection and may transmit the virus to vulnerable patients [1].

Pregnant women require particular attention because influenza can lead to severe maternal illness and adverse pregnancy outcomes. Early assessment and appropriate management are important when influenza is suspected in pregnancy. Healthcare workers should follow national clinical guidance and avoid delaying assessment of high-risk patients.

Preventing influenza and reducing transmission

Vaccination is the most effective specific preventive measure against influenza. Seasonal influenza vaccines are updated in response to changes in circulating viruses. Annual vaccination is particularly important for groups at increased risk of severe disease and for healthcare workers, in accordance with national recommendations and vaccine availability [1,4].

Everyday preventive practices also contribute to reducing transmission. Members of the public should cover their mouth and nose when coughing or sneezing, dispose of used tissues safely, clean their hands regularly and improve ventilation in indoor spaces. People with respiratory symptoms should stay at home when possible, avoid close contact with vulnerable individuals and wear a well-fitting mask when contact with others cannot be avoided, particularly in crowded or healthcare settings [4].

Individuals should follow current public health advice when returning to normal activities after illness. Where fever has occurred, they should be recovering and fever-free for at least 24 hours without fever-reducing medication.

Early healthcare-seeking and appropriate treatment

Most otherwise healthy people with influenza recover with adequate rest, fluids and supportive care. However, urgent medical assessment is necessary for difficulty breathing, chest pain, confusion, severe weakness, signs of dehydration or worsening illness.

People at increased risk of complications should contact a healthcare professional promptly when influenza is suspected. Antiviral treatment may be indicated, particularly for high-risk patients or those with severe or progressive illness, and is most beneficial when initiated early according to clinical guidance [1]. Antibiotics do not treat influenza viruses and should only be used when clinically indicated for a suspected or confirmed bacterial infection. Medicines should not be used without appropriate professional advice.

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The importance of surveillance and public participation

Effective influenza prevention depends on both individual responsibility and a coordinated public health response. Sentinel surveillance enables the Epidemiology Unit to monitor trends in respiratory illness, while laboratory testing helps identify circulating influenza viruses. Timely reporting, complete clinical and laboratory data, and communication between hospitals, laboratories and public health authorities are essential to ensure that surveillance findings inform action.

Healthcare institutions should maintain appropriate infection prevention and control measures, recognize suspected influenza early, collect specimens according to national guidance and report relevant cases through established surveillance systems. The public should follow reliable information issued by the Ministry of Health and seek advice from qualified healthcare professionals rather than relying on unverified information.

Conclusion

Influenza remains a preventable cause of respiratory illness that can have serious consequences for vulnerable populations. Public awareness, seasonal vaccination, respiratory hygiene, early recognition of severe illness and timely access to appropriate healthcare are central to reducing its impact. In Sri Lanka, continued strengthening of national influenza surveillance and effective communication between health authorities, healthcare institutions and the public are vital for timely detection and response.

" Protect yourself. Protect your family. Help prevent the spread of influenza".

Compiled By:

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Table 1 : Water Quality Surveillance
Number of microbiological water samples August 2026

District	MOH areas	No: Expected *	No: Received
Colombo	18	108	**NR**
Gampaha	15	90	**10**
Kalutara	13	78	**NR**
Kalutara NIHS	2	12	**NR**
Kandy	23	138	**NR**
Matale	13	78	**NR**
Nuwara Eliya	13	78	**40**
Galle	20	120	**NR**
Matara	17	102	**0**
Hambantota	12	72	**52**
Jaffna	14	84	**121**
Kilinochchi	4	24	**8**
Mannar	5	30	**21**
Vavuniya	4	24	**NR**
Mullatvu	6	36	**NR**
Batticaloa	14	84	**47**
Ampara	7	42	**32**
Trincomalee	12	72	**NR**
Kurunegala	29	174	**NR**
Puttlam	13	78	**18**
Anuradhapura	23	138	**0**
Polonnaruwa	9	54	**NR**
Badulla	16	96	**NR**
Moneragala	11	66	**19**
Rathnapura	20	120	**NR**
Kegalle	11	66	**92**
Kalmunai	13	78	**0**

* No of samples expected (6 / MOH area / Month)
NR = Return not received

References:

1. World Health Organization. Influenza (seasonal). Geneva: WHO; 2025. Available from: https://www.who.int/news-room/fact-sheets/detail/influenza-%28seasonal%29

2. Epidemiology Unit, Ministry of Health, Sri Lanka. Influenza – How to be prepared. Colombo: Ministry of Health. Available from: https://www.epid.gov.lk/influenza-how-to-be-prepared

3. Ministry of Health, Sri Lanka. Annual Health Bulletin 2022–2023. Colombo: Ministry of Health; 2025. Section 5.5: Influenza.

4. World Health Organization. How can I avoid getting the flu? Geneva: WHO; 2026. Available from: https://www.who.int/news-room/questions-and-answers/item/how-can-I-avoid-getting-the-flu

Table 1: Distribution of Notified Diseases reported by Medical Officers of Health 14th – 20th Sep 2026 (38th Week)

RDHS	Dengue Fever		Dysentery		Encephalitis		En. Fever		F. Poison-		Leptospirosis		Typhus		Viral Hep.		H. Rabies		Chickenpox		Meningitis		Leishman.		Tuberculosis		Leprosy		WRCD	
	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	T*	C**
Colombo	266	19593	1	19	0	2	1	5	0	21	2	239	0	2	1	18	0	0	11	422	4	89	0	5	40	1408	6	125	**26**	**100**
Gampaha	203	16266	2	37	2	36	0	0	2	27	8	381	0	6	3	15	0	0	19	624	4	229	0	54	22	784	3	59	**22**	**100**
Kalutara	97	7870	1	36	0	4	0	11	1	64	11	332	0	18	0	24	0	0	16	672	2	108	0	15	0	406	4	78	**49**	**100**
Kandy	54	6628	0	41	0	9	0	7	0	38	2	148	0	27	0	19	0	0	4	507	1	102	0	43	23	402	0	19	**42**	**100**
Matale	20	2371	0	20	0	5	0	2	0	7	0	160	0	3	0	9	0	0	2	172	0	68	5	386	3	140	0	27	**73**	**100**
Nuwara Eliya	10	1126	1	41	0	6	0	3	0	22	3	230	1	43	0	16	0	0	2	371	4	163	0	2	6	194	0	9	**73**	**100**
Galle	66	6255	2	32	1	14	1	9	0	60	8	427	1	34	2	27	0	0	9	782	1	136	0	2	18	337	0	48	**57**	**100**
Hambantota	36	2848	1	49	0	2	0	2	0	47	3	211	1	29	0	14	0	0	3	178	0	55	8	409	4	109	0	22	**92**	**100**
Matara	24	6495	0	29	0	4	0	3	2	14	8	305	0	20	0	17	0	0	2	406	2	144	1	109	1	120	1	27	**24**	**100**
Jaffna	23	1102	5	81	1	7	0	20	6	102	0	56	2	245	0	26	0	0	0	291	3	47	0	1	4	138	0	12	**94**	**100**
Kilinochchi	4	752	1	14	0	2	0	8	0	1	0	86	0	12	0	3	0	1	2	104	0	18	0	2	1	28	0	5	**47**	**100**
Mannar	2	551	0	1	0	3	0	0	0	3	0	47	0	4	0	3	0	0	1	66	0	21	0	5	2	41	0	5	**28**	**100**
Vavuniya	0	371	0	22	0	4	0	2	5	13	0	110	0	4	0	1	0	0	1	138	1	36	0	24	2	54	0	7	**9**	**100**
Mullaitivu	0	112	0	6	0	2	0	1	0	13	0	37	0	6	0	2	0	0	0	13	0	10	0	10	0	26	0	7	**65**	**100**
Batticaloa	23	1602	1	68	1	15	0	2	2	23	0	149	0	3	3	19	0	0	6	268	1	79	0	24	0	117	4	88	**73**	**100**
Ampara	11	685	1	54	0	2	0	1	0	16	3	132	0	2	0	17	0	0	5	284	1	65	0	17	2	44	3	25	**57**	**100**
Trincomalee	6	770	0	25	0	6	0	2	0	15	1	80	0	8	0	4	0	0	5	194	0	32	0	22	6	114	0	7	**100**	**100**
Kurunegala	23	3469	0	23	0	18	0	3	0	66	6	326	1	32	0	13	0	1	6	620	2	300	2	332	6	241	1	54	**47**	**100**
Puttalam	11	1896	0	19	0	11	0	0	0	9	3	201	0	26	0	8	1	5	1	132	1	142	0	19	0	141	0	24	**46**	**93**
Anuradhapura	17	1149	0	19	0	9	0	3	1	54	2	207	0	26	1	14	0	0	10	458	4	129	22	598	9	203	1	51	**14**	**100**
Polonnaruwa	3	836	0	25	0	7	0	2	1	60	0	226	0	7	4	30	0	1	4	389	1	59	3	399	4	67	0	67	**90**	**90**
Badulla	22	1752	2	36	0	8	0	3	0	13	2	210	6	50	1	121	0	0	2	351	3	209	6	105	9	196	0	15	**49**	**100**
Monaragala	18	1308	0	15	0	3	0	2	1	12	1	228	0	31	2	58	0	1	1	192	1	88	0	173	4	100	0	19	**80**	**100**
Ratnapura	44	5407	0	31	0	8	2	9	2	23	26	752	0	32	0	15	0	0	4	297	1	92	9	170	9	304	1	32	**50**	**100**
Kegalle	24	2811	0	47	0	6	0	5	0	29	5	327	1	12	0	13	0	0	5	573	1	81	0	13	5	242	2	11	**61**	**100**
Kalmunai	42	1079	0	44	0	3	0	1	0	22	1	64	0	1	0	3	0	0	3	387	0	36	0	0	2	92	0	26	**53**	**100**
SRILANKA	**1049**	**95104**	**18**	**834**	**5**	**196**	**4**	**106**	**23**	**774**	**95**	**5671**	**13**	**683**	**17**	**509**	**1**	**9**	**124**	**8891**	**38**	**2538**	**56**	**2939**	**182**	**6048**	**26**	**869**	**55**	**99**

Source: WRCD module of the EPINET.
A = Cases reported during the current week;
T*=Timeliness refers to returns received on or before 20th Sep, 2026.
B = Cumulative cases for the year.
Total number of reporting units 363
C**-Completeness;
C**-Completeness;

Table 2: Selected Vaccine Preventable Diseases & AFP 14th – 20th Sep 2026 (38th Week)

Disease	No. of Cases by Province									Number of cases during current week in 2026	Number of cases during same week in 2025	Total number of cases to date in 2026	Total number of cases to date in 2025	Difference between the number of cases to date in 2026 & 2025
	W	C	S	N	E	NW	NC	U	Sab					
AFP[1]	00	00	00	00	00	00	00	00	00	00	02	45	47	-4.2%
Diphtheria	00	00	00	00	00	00	00	00	00	00	00	00	00	0 %
Mumps[2]	00	00	01	01	00	00	00	00	00	02	01	108	110	-1.8%
Measles[3]	00	00	00	00	00	02	01	00	00	03	00	63	02	3050 %
Rubella[3]	00	00	00	00	00	00	00	00	00	00	00	00	00	0 %
CRS[2]	00	00	00	00	00	00	00	00	00	00	00	00	00	0 %
Tetanus[2]	00	00	00	00	00	00	00	00	00	00	00	04	04	0 %
Neonatal Tetanus[2]	00	00	00	00	00	00	00	00	00	00	00	00	00	0 %
Japanese Encephalitis[3]	00	00	00	00	00	00	00	00	00	00	00	00	04	-100 %
Whooping Cough[2]	00	00	00	00	00	00	00	00	01	01	00	22	16	37.5 %

Key to Table 2

Provinces: **W**: Western, **C**: Central, **S**: Southern, **N**: North, **E**: East, **NC**: North Central, **NW**: North Western, **U**: Uva, **Sab**: Sabaragamuwa.

Data Sources:
Weekly Return of Communicable Diseases: Diphtheria, Mumps, Tetanus, Neonatal Tetanus, Whooping Cough.
Special Surveillance: AFP, Measles, Rubella, CRS.

AFP[1] = No Polio cases
Mumps[2], CRS[2], Tetanus[2], Neonatal Tetanus[2], Whooping Cough[2]—Clinically and/ or laboratory confirmed cases
Measles[3], Rubella[3], Japanese Encephalitis[3]— Laboratory Confirmed cases

AFP—Acute Flaccid Paralysis
CRS = Congenital Rubella Syndrome
NA = Not Available

AFP and all Vaccine Preventable Diseases should be investigated by the MOH Personally.

All patients presenting with fever and a maculopapular rash must be promptly notified and investigated. Blood samples and throat swabs should be collected at the appropriate time window

Comments and contributions for publication in the WER Sri Lanka are welcome. However, the editor reserves the right to accept or reject items for publication. All correspondence should be mailed to The Editor, WER Sri Lanka, Epidemiology Unit, P.O. Box 1567, Colombo or sent by E-mail to **chepid@sltnet.lk.** The Epidemiology Unit should be formally acknowledged in all resulting publications as the primary data source.

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ON STATE SERVICE

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